

How to Open an In-Home Care Agency in Wisconsin

A step-by-step guide to Wisconsin's two paths — private-pay supportive home care, and the Personal Care Agency certification you need to bill Medicaid — plus screening caregivers, finding clients, and getting paid

PolicyReady.org — Practical Playbooks for Care Entrepreneurs

The Wisconsin difference — read this first

Wisconsin has two paths, and which one you take depends on how you get paid. If you provide only companionship and homemaker help — what Wisconsin calls supportive home care — and clients pay privately, you generally do not need a state license to open. But the moment you want to provide hands-on personal care billed to Medicaid, you need a Personal Care Agency (PCA) certification from the Wisconsin Department of Health Services, Division of Quality Assurance (DQA), under Wis. Admin. Code s. DHS 105.17. Wisconsin treats Medicaid personal care as physician-ordered, registered-nurse-supervised care delivered under a written care plan — so a certified PCA is built around an RN. This guide walks you through both paths, the DQA certification, the caregiver background checks Wisconsin is strict about, how you find clients, and how you get paid — privately and through Wisconsin Medicaid, Family Care, and IRIS.

How to Use This Guide

This guide takes you from idea to an operating Wisconsin in-home care agency. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Wisconsin agency owners — not legal, tax, or licensing advice. Requirements, fees, and phone numbers change, so verify each step with the agency named before you rely on it. Where this guide gives a figure, treat it as a starting point to confirm, not a final answer.

The one-paragraph version

Decide your path. If you will serve only private-pay companionship and homemaker clients, form a Wisconsin business — get an EIN, insure it, run caregiver background checks, and you can generally open without a state license. If you will bill Medicaid for personal care, you also apply to DQA for a Personal Care Agency certification: submit your application and proposal through the DHS DQA Provider Portal, pass an on-site survey to s. DHS 105.17 and s. DHS 107.112, get recommended for Medicaid certification, and enroll with Wisconsin Medicaid (ForwardHealth). Either way, build your team: every caregiver clears a Wisconsin caregiver background check, completes your training, and is found competent. A certified PCA delivers personal care on a physician's order, under registered-nurse supervision, with an RN home visit at least every sixty days — so line up your RN early.

The Path at a Glance

Ten steps take you from idea to an operating Wisconsin in-home care agency:

1. **Choose your path.** Private-pay supportive home care (no state license) or a Medicaid-certified Personal Care Agency (DQA certification) — or both.

2. **Form the business.** Register with the Wisconsin Department of Financial Institutions; get an EIN; open a business bank account.
3. **Name an administrator and line up an RN.** A certified PCA runs on registered-nurse supervision — secure your RN supervisor early.
4. **Put protection in place.** Professional and general liability insurance and workers' compensation.
5. **Write your policies.** Adopt the policy and procedure manual, client forms, and caregiver handbook built to DHS 105.17 and DHS 107.112.
6. **Apply to DQA (for Medicaid).** Submit your PCA application and proposal through the DHS DQA Provider Portal.
7. **Pass the DQA survey.** An on-site survey to s. DHS 105.17 and s. DHS 107.112; then DQA recommends Medicaid certification.
8. **Screen and train caregivers.** Wisconsin caregiver background checks (DHS 12), orientation, training, and RN-verified competency.
9. **Enroll and start serving.** Enroll with ForwardHealth, set up EVV, and begin — on physician orders and care plans, under RN supervision.
10. **Add payers and grow.** Layer in Family Care and IRIS, build referral relationships, and expand.

Part 1 — What This Business Is

An in-home care agency sends caregivers into clients' homes to help with everyday life: bathing, dressing, grooming, toileting, transferring, and eating (personal care); cleaning, laundry, meals, and errands (homemaker and supportive services); companionship; and accompanying clients to medical appointments. In Wisconsin, how you deliver and bill for that care determines what credential you need — which is the first thing to get straight.

Wisconsin is a strong market for in-home care: a large and growing senior population, higher-than-average private-pay rates in the Madison and Milwaukee areas, and a well-developed Medicaid long-term-care system through Family Care and IRIS. The trade-off is that serving Medicaid clients means becoming a certified Personal Care Agency and operating under registered-nurse supervision — more structure up front, but access to a much larger client base and a credential that gives you credibility with hospitals, care managers, and families.

A useful way to think about it: the private-pay side of the business is a service company, and the Medicaid-certified side is a clinical operation wrapped around a registered nurse. Many successful Wisconsin owners run both at once — private-pay clients keep cash flowing while the certified side reaches the larger Medicaid population. Deciding which you are building first tells you what to prioritize in your first ninety days: speed and marketing for private pay, or your RN, your policies, and your DQA application for the certified path.

The service lines, in plain terms

- **Personal care:** hands-on help with the activities of daily living — the core of a certified PCA, billable to Medicaid.
- **Supportive home care:** companionship and homemaker help (housekeeping, laundry, meals) — often private pay, and generally no state license required.
- **Instrumental support and errands:** shopping, meal prep, and accompanying clients to medical appointments, as the care plan directs.
- **What you don't do:** skilled nursing and therapy — those need a separate home health agency license under ch. DHS 133.

Part 2 — Wisconsin's Two Paths

Here is the most important thing to understand about opening in Wisconsin: there is no single "home care license." What you need depends on the services you provide and how you bill.

This is the single most common point of confusion for people opening in Wisconsin, and getting it right early saves months. Ask yourself two questions: what services will I provide, and who will pay for them? If the honest answer is companionship and homemaker help paid privately, you are on Path A and can move quickly. If it is hands-on personal care paid by Medicaid, you are on Path B and need the DQA certification and an RN. Most owners eventually want both, so it is worth understanding each path even if you start with one.

Path A — Private-pay supportive home care

If your agency provides only companionship and homemaker services — supportive home care — and clients pay privately, Wisconsin generally does not require a state license to open. You still form a business, carry insurance, run caregiver background checks, and train your staff, but there is no DQA application or survey to clear first. This is the fastest, lowest-cost way in, and many agencies start here.

Path B — A Medicaid-certified Personal Care Agency

To provide hands-on personal care billed to Wisconsin Medicaid, you need a Personal Care Agency (PCA) certification from DQA under s. DHS 105.17. A freestanding personal care agency is one of the entity types that can be certified. This is where the real regulatory structure lives: personal care is provided on a physician's written order, under registered-nurse supervision, and according to a written care plan — with an RN home visit at least every sixty days during which the nurse directly observes your caregiver providing care.

That RN-centered structure is not red tape for its own sake — it is what lets Wisconsin treat personal care as a Medicaid-funded health service. The physician order establishes medical need, the RN assessment and care plan translate it into concrete tasks, and the sixty-day supervisory visit confirms the care is actually being delivered as planned. If you understand and build around that spine, the rest of the certification falls into place; if you fight it, the survey will be hard. Treat your RN not as a compliance checkbox but as the clinical leader of your agency.

Most agencies want Path B — because that's where the clients are

Medicaid pays for the majority of long-term in-home care in Wisconsin, through Medicaid personal care and the Family Care and IRIS programs. A private-pay-only agency can open faster, but it is limited to families who can pay out of pocket. Getting certified as a PCA opens the far larger Medicaid market — so even agencies that start private-pay usually pursue PCA certification. Because a certified PCA runs on registered-nurse supervision, line up your RN supervisor before you apply.

Part 3 — Getting Certified by DQA

If you are taking Path B, you apply for Personal Care Agency certification through the Wisconsin Department of Health Services, Division of Quality Assurance (DQA). It is a three-step process, and it centers on showing DQA that you can deliver personal care safely under registered-nurse supervision.

1. **Apply through the DHS DQA Provider Portal.** Submit your application and a proposal to provide personal care services, along with your policies and procedures, and the applicable fee.
2. **Pass an on-site DQA survey.** DQA reviews your agency against s. DHS 105.17 and s. DHS 107.112 — your administration, RN supervision, care planning, caregiver screening and training, and documentation.
3. **Get recommended for Medicaid certification, then enroll.** On a successful survey, DQA recommends approval to Wisconsin Medicaid; you then enroll with ForwardHealth to bill.

Build to the rule, and keep it current

Wisconsin DQA surveyors expect policies that reference s. DHS 105.17 and s. DHS 107.112 specifically — generic or other-state policies won't pass. Build your policy manual and your caregiver and client files to those rules from the start, keep your certification and ForwardHealth enrollment current, keep your caregiver background checks and RN supervisory visits on schedule, and stay ready for a DQA survey or complaint investigation at any time. A branch office is separately certified if it can't adequately share supervision with your main office.

Part 4 — The RN Supervisor & Care Planning

A Wisconsin certified PCA is built around a registered nurse. This is the biggest structural difference from a simple private-pay agency, so plan for it from day one.

- **Physician orders.** Medicaid personal care is provided on the client's physician's written order — obtained before care begins and renewed as required.
- **RN assessment and care plan.** Your registered nurse assesses the client and develops a written care plan of personal care tasks that your caregivers follow.
- **The sixty-day supervisory visit.** Your RN visits the client's home at least every sixty days, at a time when your caregiver is directly observed providing care, and documents the visit.
- **Retraining when needed.** If the RN sees a caregiver not following the care plan, you provide counseling, education, or retraining.
- **A narrow waiver.** A non-Medicaid client may waive the sixty-day visit by a written agreement that becomes part of the service agreement — Medicaid clients cannot.

Because everything runs through the RN, secure your registered-nurse supervisor early — whether employed or contracted — and build your scheduling around the sixty-day supervisory visits so none slip.

A practical tip: track every client's next supervisory-visit due date the same way you track payroll — as a hard deadline, not a soft goal. A missed sixty-day visit is one of the easiest deficiencies for a surveyor to spot and one of the hardest to explain, because the date math is right there in the file. Some agencies schedule the next visit at the end of the current one, so the calendar never has a gap. The same discipline applies to renewing physician orders and updating care plans when a client's condition changes.

Part 5 — Caregiver Background Checks & Training

Wisconsin's caregiver background-check program is one of the strictest in the Midwest, and it applies whichever path you take.

- **Caregiver background checks (DHS 12).** Complete a Wisconsin caregiver background check on every person who provides direct care before they have regular contact with clients, and repeat it on the required schedule. A person barred under the Caregiver Program can't provide care.
- **Caregiver misconduct (DHS 13).** Check the caregiver misconduct registry before hire, and report and investigate any alleged caregiver misconduct to DQA.
- **Orientation and training.** Prepare every personal care worker before independent client contact — the care plan, client rights, abuse reporting, safe personal care, infection control, documentation, and EVV.
- **RN-verified competency.** Your registered nurse verifies competency by directly observing the worker during supervisory visits — screening, training, and RN observation together keep your caregivers ready.

Document everything, in every file

Wisconsin's screening and your training and supervision are only as good as your records. Document each caregiver's background check and its determination, their orientation and training, and each RN supervisory observation in their personnel file. At a survey, DQA will expect to see that every caregiver was screened before client contact, trained for their tasks, and observed by your RN.

Part 6 — Setting Up to Operate

Form the business, name an administrator, and secure an RN

Register your business with the Wisconsin Department of Financial Institutions, get a federal EIN, and open a business bank account. Name an administrator responsible for running the agency and its compliance, and — for a certified PCA — secure your registered-nurse supervisor. Give both roles real authority and time.

Insurance, policies, and enrollment

Carry professional and general liability insurance and workers' compensation coverage. Adopt a policy and procedure manual, client forms, and a caregiver handbook built to s. DHS 105.17 and s. DHS 107.112. For Medicaid, enroll with ForwardHealth after DQA recommends certification, set up Electronic Visit Verification, and decide whether to contract with Family Care managed care organizations and participate in IRIS. Your physician orders, care plans, and RN supervision are your quality-control system.

Think of your administrator and your RN as the two load-bearing roles of the agency. The administrator owns the business — enrollment, insurance, records, billing, hiring, and the DQA relationship — and the RN owns the clinical side — assessments, care plans, supervision, and caregiver competency. In a small startup one person may wear more than one hat, but the responsibilities still have to be covered, and both roles need enough authority and time to do the job. Underfunding either one is where new agencies most often get into trouble.

Part 7 — Finding Clients & Referrals

Two engines fill your schedule: families who pay privately, and the aging-and-disability network — including the Family Care and IRIS system — that steers people to agencies. Build both from the start.

- **Hospital discharge planners and case managers** send people home who need help — be the certified agency they trust to prevent a readmission.
- **Family Care managed care organizations and IRIS consultants** authorize and coordinate long-term care — your key Medicaid referral and payer partners.
- **Skilled nursing and rehab facilities** discharge clients who still need in-home support after therapy ends.
- **Hospices** refer both ways: they need aides for personal care, and you refer declining clients to them.

- **Assisted living and senior communities** have residents who need one-on-one help beyond what the community provides.
- **Aging & Disability Resource Centers (ADRCs), elder-law attorneys, and care managers** guide Wisconsin families to trusted agencies.

Your PolicyReady Referral System for Wisconsin lists named hospitals, skilled nursing facilities, hospices, senior communities, elder-law attorneys, and care managers across Milwaukee, Madison, Green Bay, Kenosha, and Appleton, with the playbook for turning them into a steady stream of clients.

Referral relationships are earned, not announced. The agencies and professionals who send you clients are putting their own reputation on the line, so they refer to providers they trust to be reliable, responsive, and compliant. Show up on time, communicate clearly, handle problems without drama, and make the referrer look good to the family — and the referrals compound. In Wisconsin, the Family Care managed care organizations and IRIS consultants are especially worth the investment, because they sit at the center of the Medicaid long-term-care system and can send steady, authorized volume once they know your agency delivers.

Part 8 — Getting Paid

In-home care in Wisconsin is paid three main ways: privately by the client or family; by Wisconsin Medicaid through Medicaid personal care and the long-term-care programs Family Care and IRIS; and by other sources such as long-term-care insurance and Veterans benefits. Private-pay agencies can start fastest; a certified PCA reaches the much larger Medicaid market.

Private pay

Set a clear hourly rate, put it in a written service agreement, and bill only for services actually provided. Private pay needs no Medicaid certification for supportive home care, and rates in the Madison and Milwaukee areas tend to run higher than in rural parts of the state. Build in your real labor costs, including overtime and Wisconsin's minimum wage.

Wisconsin Medicaid — personal care, Family Care, and IRIS

To bill Medicaid, you must be a certified PCA enrolled with ForwardHealth. Medicaid funds personal care directly, and through two long-term-care programs: Family Care, where a managed care organization arranges and authorizes a member's services, and IRIS (Include, Respect, I Self-Direct), where the participant directs their own budget and hires their own workers. You deliver only authorized personal care on a physician order and care plan, capture Electronic Visit Verification on every Medicaid visit, and bill through ForwardHealth.

The practical takeaway is that Medicaid billing rewards discipline. Authorized services, a current physician order, a written care plan, an EVV-verified visit, and clean documentation that all line up are what get claims paid and keep you clear in an audit. Build that chain into your daily workflow from your first Medicaid client, and reconcile your EVV, your visit notes, and your billing regularly so small mismatches are caught early rather than at a payer review.

EVV is required — set it up before you bill Medicaid

Wisconsin requires Electronic Visit Verification for Medicaid personal care: your caregiver checks in and out at the client's location so the visit is verified electronically. Set up EVV and train your caregivers on it before you take Medicaid clients, and reconcile your EVV records to your documentation and billing so claims are clean.

Part 9 — Your First 90 Days

1. **Before you apply:** Choose your path, form the business, get your EIN and bank account, name your administrator, secure your RN supervisor, and bind insurance and workers' comp.
2. **Before you apply:** Adopt your policy manual, client forms, and caregiver handbook to s. DHS 105.17 and s. DHS 107.112, and build your caregiver background-check and training systems.
3. **Apply to DQA (Path B):** Submit your PCA application and proposal through the DHS DQA Provider Portal and prepare for the on-site survey.
4. **During review:** Recruit caregivers, complete their background checks and training, and get ready for the survey; line up your first physician relationships.
5. **After certification:** Enroll with ForwardHealth, set up EVV, take your first clients on physician orders and care plans, run your RN supervisory visits, and start referral outreach to a Family Care MCO and two or three hospitals or hospices.

Part 10 — Common Mistakes to Avoid

- **Assuming you need one "home care license."** Wisconsin has two paths — private-pay supportive home care (no license) and a Medicaid-certified PCA (DQA certification). Match the credential to how you bill.
- **Waiting to line up an RN.** A certified PCA runs on registered-nurse supervision — secure your RN before you apply, not after.

- **Billing Medicaid without a physician order or care plan.** Medicaid personal care requires both — and RN supervision.
- **Missing the sixty-day supervisory visit.** Build your scheduling around it so none slip; only a non-Medicaid client can waive it, in writing.
- **Skipping or delaying caregiver background checks.** Complete the Wisconsin caregiver background check before any regular client contact.
- **Skipping EVV setup.** Every Medicaid personal care visit must be captured through EVV; set it up before you bill.
- **Providing skilled care beyond your certification.** Skilled nursing and therapy require a separate home health agency license — otherwise refer it out.

Part 11 — Staying Compliant & Growing

Once you are open, compliance is ongoing: keep your certification and ForwardHealth enrollment current; keep every caregiver's background check, training, and RN-verified competency current; keep physician orders and care plans updated and RN supervisory visits on schedule; maintain insurance; and run a quality program that tracks complaints, incidents, and satisfaction and fixes what they show. Keep a compliance calendar so nothing lapses, and stay ready for a DQA survey at all times.

The agencies that stay out of trouble treat compliance as a weekly habit rather than an annual scramble. A short recurring review — which caregivers are due for a background-check refresh, which clients are due for a supervisory visit or an updated order, which training is coming due — catches small lapses before they become survey findings. Your registered nurse and your administrator should own this review together, because the clinical and the administrative deadlines are intertwined. Build the habit while you are small and it will scale with you.

Growth comes from three moves: deepen referral relationships — especially with Family Care MCOs and IRIS consultants — until you are the agency they call first; add payers (layer Medicaid, Family Care, and IRIS onto private pay, then long-term-care insurance and VA benefits); and expand your service area and team at a pace your administrator and RN supervision can actually support. As you grow, don't let volume outrun your background checks, your RN supervisory visits, or your documentation — those are the things a survey looks at hardest, and the things that protect your clients.

Part 12 — What It Costs to Start

Your startup budget depends on your path. A private-pay supportive home care agency is lighter — business formation, insurance, background checks, and policies. A certified PCA adds the DQA application and the cost of registered-nurse supervision. Plan for the following, and gather real quotes rather than guessing:

- **DQA certification (Path B):** the PCA application through the DHS DQA Provider Portal, with the applicable fee — confirm the current amount with DQA.
- **Registered-nurse supervision (Path B):** an employed or contracted RN to assess clients, build care plans, and conduct the sixty-day supervisory visits — often the largest ongoing cost that distinguishes a PCA.
- **Insurance:** professional and general liability and workers' compensation — get quotes for your size and services.
- **Business formation:** registration with the Wisconsin Department of Financial Institutions, plus your accounting setup.
- **Policies and documents:** your policy and procedure manual, client forms, and caregiver handbook built to s. DHS 105.17 and s. DHS 107.112.
- **Caregiver screening, training, and systems:** Wisconsin caregiver background checks, orientation and training, scheduling and documentation software, and EVV for Medicaid.
- **Working capital:** enough cash to cover several months of wages, RN supervision, insurance, and overhead before revenue is steady — the cost new owners most often underestimate.

In-home care is not equipment-heavy, which keeps startup lighter than facility-based care, but for a certified PCA the working-capital runway to certification and your first Medicaid payments is real — fund it deliberately, and remember the RN is an ongoing cost, not a one-time one.

One more piece of budgeting advice: separate your one-time startup costs from your ongoing monthly costs, and make sure you have funded both. The application, business formation, and initial policy setup are one-time; wages, RN supervision, insurance, software, and background-check renewals are monthly and recur whether or not your schedule is full. New owners who plan only for the startup costs and not for the months of payroll before revenue catches up are the ones who run short. A realistic runway — several months of full operating costs in reserve — is what carries you from your first client to a stable, growing agency.

Two Worked Examples

Example 1 — A private-pay start in Milwaukee

You want to open quickly, so you start on Path A. You form a Wisconsin LLC, name an administrator, bind insurance and workers' comp, and adopt your policies. You hire two caregivers, complete their Wisconsin caregiver background checks, and train them on supportive home care. Your first client comes from a hospital discharge planner whose patient needs companionship, meal prep, and light housekeeping at home after a hospital stay — private pay, billed weekly. As demand grows and families ask about Medicaid, you begin the DQA certification process so you can serve Medicaid personal care clients too. Referral targets you would build in Milwaukee include hospitals, skilled nursing and rehab facilities, hospices, and senior communities across the metro.

Example 2 — A certified PCA in Madison or Green Bay

You are going straight for Medicaid, so you take Path B. You secure a registered-nurse supervisor, form the business, and adopt policies built to s. DHS 105.17 and s. DHS 107.112. You submit your PCA application and proposal through the DHS DQA Provider Portal, pass the on-site survey, and are recommended for Medicaid certification; you enroll with ForwardHealth and set up EVV. A Family Care managed care organization or an IRIS consultant authorizes personal care for a member; your RN assesses the client, obtains the physician order, and writes the care plan; you assign a screened, trained caregiver; and your RN conducts the sixty-day supervisory visits. As you take Medicaid clients, you keep building the private-pay side through the same referral network — hospitals, skilled nursing, hospices, assisted living, and the ADRCs — in Madison, Green Bay, and the surrounding communities.

The lesson of this second example is that the certified path front-loads the work — the RN, the policies, the application, the survey — but opens the door to the Medicaid population that makes up the bulk of long-term in-home care. Owners who go straight for certification are trading a slower start for a larger and more durable client base, anchored by relationships with the Family Care MCOs and IRIS consultants who authorize the care.

Who to Call & What to Ask

For...	Who to contact
Forming your business	Wisconsin Department of Financial Institutions (wdfi.org). Ask: what do I file to register my LLC, and what are the ongoing requirements?
Certifying your agency (Medicaid)	Wisconsin DHS, Division of Quality Assurance (DQA) — Personal Care Agency certification via the DHS DQA Provider Portal (dhs.wisconsin.gov , Personal Care Agencies). Ask: what does a complete PCA application and proposal require, and what are the current fee and survey timeframes?
Caregiver background checks	Wisconsin Caregiver Program, DHS (Caregiver Program Manual P-00038; ch. DHS 12 and DHS 13). Ask: how do I complete caregiver background checks and check the misconduct registry?
Enrolling as a Medicaid provider	Wisconsin Medicaid / ForwardHealth (forwardhealth.wi.gov). Ask: how do I enroll as a personal care provider and set up EVV?
Family Care & IRIS	Your regional Family Care managed care organizations and IRIS consultant agencies, and your Aging & Disability Resource Center (ADRC). Ask: how do I contract or coordinate to serve members?
The aging & disability network	Your Aging & Disability Resource Center (ADRC) and the Eldercare Locator (1-800-677-1116). Ask: how do families and Medicaid clients find and choose in-home care?
Wage & hour rules	Wisconsin Department of Workforce Development and the U.S. Department of Labor. Ask: what are the current Wisconsin minimum wage and the FLSA home care overtime

For...	Who to contact
	rules?
Reporting abuse	The county adult-at-risk / adult protective services agency (Wis. Stat. ch. 55; s. 46.90); county child protective services; 911 for emergencies; report caregiver misconduct to DQA.

Key Terms

- **Personal Care Agency (PCA) certification:** the DQA certification a freestanding agency needs to bill Medicaid for personal care, under s. DHS 105.17.
- **Supportive home care:** private-pay companionship and homemaker services that generally need no state license.
- **DQA:** the Wisconsin DHS Division of Quality Assurance, which surveys and certifies personal care agencies.
- **ForwardHealth:** Wisconsin Medicaid's provider system, where a certified PCA enrolls to bill.
- **Family Care:** Wisconsin's managed long-term-care program; an MCO arranges and authorizes services.
- **IRIS (Include, Respect, I Self-Direct):** the self-directed long-term-care program where the participant directs their budget and hires workers.
- **Physician order & care plan:** Medicaid personal care is provided on a physician's written order and a written care plan.
- **RN supervisory visit:** the registered-nurse home visit — at least every sixty days — that directly observes the caregiver providing care.
- **Caregiver background check (DHS 12):** Wisconsin's required background check before a caregiver has regular client contact.
- **EVV:** Electronic Visit Verification, required for Medicaid personal care visits.

Core Rules & References

- Wis. Admin. Code s. DHS 105.17 — personal care providers (Medicaid certification of personal care agencies)
- Wis. Admin. Code s. DHS 107.112 — personal care services (covered services, physician order, care plan, RN supervision)
- Wis. Admin. Code ch. DHS 12 — caregiver background checks; ch. DHS 13 — reporting and investigation of caregiver misconduct
- Wis. Stat. s. 50.49 and ch. DHS 133 — home health agency licensing (skilled services — a separate license, not required for personal care)
- Wis. Stat. ch. 55 and s. 46.90 — protection of adults at risk and elder adults at risk (abuse and neglect reporting)
- Wisconsin Medicaid (ForwardHealth); Family Care and IRIS; Electronic Visit Verification under the 21st Century Cures Act

This guide is a general educational resource for prospective Wisconsin in-home care agency owners. It is not legal, tax, or licensing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with the agency named — the Wisconsin Department of Financial Institutions, the DHS Division of Quality Assurance, the Wisconsin Caregiver Program, and the relevant payer — before you rely on it. Provisions cited reflect Wisconsin law and DHS rules at the time of writing.